

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: Treatment Plan for Managed Access Program (MAP) to Provide Access to Ruxolitinib, for Patients With Polycythemia Vera (PV)

Short Title/Acronym: MAP to Provide Access to Ruxolitinib, for Patients With Polycythemia Vera

Protocol Number: CINC424B2002I

NCT Number: NCT05269771

Sponsor/Company Name: Novartis

Investigational Product: Ruxolitinib (JAK1/JAK2 inhibitor) |

Trade Names: Jakafi, Opzelura | **ATC Code:** D11AH09

Phase of Development: Not Applicable (Expanded Access / Managed Access Program)

Medical Monitor: Novartis Pharmaceuticals Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To provide access to ruxolitinib for eligible patients diagnosed with Polycythemia Vera who have no comparable or satisfactory alternative therapy available and cannot participate in a standard clinical trial.

Secondary Objectives: To monitor and evaluate the general safety, tolerability, and clinical benefit of ruxolitinib in a real-world, managed access setting.

2.2 Background & Rationale To address the critical medical need for patients with serious or life-threatening Polycythemia Vera who are not eligible for or able to participate in a clinical trial. This Cohort Treatment Plan is implemented because there is no comparable or satisfactory alternative therapy available for the diagnosis, monitoring, or treatment of these patients, and the potential patient benefit justifies the potential risks of the treatment.

3. Study Design & Methodology

3.1 Design Framework Study Type: Expanded Access (Intermediate-size Population / Cohort Treatment Plan)

Control Type: Single-arm

Blinding: Open-label

Randomization: N/A

3.2 Planned Enrollment & Duration Target \$N\$: N/A (Intermediate-size population based on independent physician requests rather

than a fixed statistical target)

Number of Sites: Global (Including Europe, Canada, Australia/NSW, and Rest of World)

Estimated Study Start: 07-Dec-2022

Estimated Primary Completion: 07-Dec-2022

Trial Status: No Longer Available

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria Patients eligible for inclusion in this Treatment Plan must meet all of the following criteria: 1. Male or female aged 12 or over. 2. Confirmed diagnosis of Polycythemia Vera (PV) based on World Health Organization (WHO) criteria. 3. Patients must have a treatment history for PV that meets the definition of resistance or intolerance to hydroxyurea (HU) or other cytoreductive therapy. 4. Patients with a peripheral blood blast count of 0% at screening. 5. Patients must have recovered or stabilized sufficiently from adverse drug reactions associated with prior treatments before beginning treatment with ruxolitinib. 6. Patients with an Eastern Cooperative Oncology Group (ECOG) score of 0, 1 or 2. Written patient informed consent must be obtained prior to start of treatment.

4.2 Exclusion Criteria Patients eligible for this Treatment Plan must not meet any of the following criteria: 1. History of hypersensitivity to any drugs or metabolites of similar chemical classes as ruxolitinib. 2. Presence of an active uncontrolled infection including significant bacterial, fungal, viral (including CMV, EBV, HHV-6, HBV, HCV, BK or HIV) or parasitic infection requiring treatment. 3. History of progressive multifocal leuko-encephalopathy (PML). 4. Presence of severely impaired renal function defined by serum creatinine > 2 mg/dL (>176.8 µmol/L), or have estimated creatinine clearance < 30 ml/min measured or calculated by Cockcroft Gault equation. 5. Women of child-bearing potential, defined as all women physiologically capable of becoming pregnant, UNLESS they are meeting specific criteria (e.g., sexual orientation precluding intercourse, sterilized partners, or using a highly effective method of birth control throughout treatment and 30 days post-treatment). 6. Pregnancy. 7. Not able to understand and to comply with treatment instructions and requirements.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Ruxolitinib dosing as recommended by the treating physician in accordance with the suggested treatment guidelines provided by Novartis.

Route of Administration: Oral

Duration of Treatment: Ongoing until lack of clinical benefit, disease progression, unacceptable toxicity, or the product becomes commercially available in the patient's region.

5.2 Concomitant & Prohibited Therapy Permitted: Standard of care therapies and medications for co-morbidities as per the treating physician's discretion.

Prohibited: Concurrent participation in another therapeutic or investigational clinical trial.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -14 to 0	Informed Consent, Medical History, Independent Physician Request & Eligibility Confirmation
Baseline	Day 0	First Dose, Baseline Safety Assessment
Interim	Week [X] to [Y]	Routine Care Visits, Safety Monitoring, SAE Reporting per physician's standard practice
End of Study	Variable	Treatment Discontinuation or Transition to Commercial Supply

(Note: Exact intervals are determined by the treating physician's standard of care under the Managed Access Program guidelines.)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Safety and tolerability profile as determined by the incidence and severity of Adverse Events (AEs) and Serious Adverse Events (SAEs) while on the Managed Access Program.

Measurement Method: Routine clinical assessments and mandatory pharmacovigilance reporting by the treating physician.

7.2 Secondary & Exploratory Endpoints

- * **Endpoint 1:** Clinical benefit and disease control (e.g., symptom management) as assessed by the treating physician.
- * **Endpoint 2:** Reason for treatment discontinuation.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Collection of AEs as per Managed Access Program regulations and local health authority requirements, monitored by the treating physician.

Serious Adverse Events (SAEs): Mandatory reporting to Novartis pharmacovigilance and local health authorities within 24 hours of physician awareness.

Data Monitoring Committee (DMC): Novartis internal safety review and pharmacovigilance teams.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (All patients who receive at least one dose of ruxolitinib through the MAP).

Sample Size Justification: N/A. The size of this expanded access program is based on individual physician requests and patient need, not formal statistical power calculations.

Handling of Missing Data: Descriptive reporting only; missing data will not be imputed as there is no formal hypothesis testing.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This program will be conducted in accordance with the general principles of Good Clinical Practice (GCP), the Declaration of Helsinki, and strict adherence to local health authority regulations for Managed Access Programs.

Monitoring Plan: Routine pharmacovigilance tracking and drug accountability monitoring by the sponsor.

Audit Trail: Standardized SAE reporting forms and accountability logs maintained by the treating physician and sponsor.

11. Study Identifiers & Governance

Primary ID: CINC424B2002I

Posting ID: 677469685605250746

Registry Number: NCT05269771

Sponsor/Lead Organization: Novartis

Study Title: MAP to Provide Access to Ruxolitinib, for Patients With Polycythemia Vera

Official Regulatory Title: Treatment Plan for Managed Access Program (MAP) to Provide Access to Ruxolitinib, for Patients With Polycythemia Vera (PV)

12. Clinical Framework (Polycythemia Vera Specific)

Primary Condition / Disease Area: Polycythemia Vera (PV)

Disease Definition: A myeloproliferative disorder of unknown etiology, characterized by abnormal proliferation of all hematopoietic bone marrow elements and an absolute increase in red cell mass and total blood volume, associated frequently with splenomegaly, leukocytosis, and thrombocythemia. Hematopoiesis is also reactive in extramedullary sites (liver and spleen). In time myelofibrosis occurs.

Preferred UMLS Name: Ruxolitinib therapy for Polycythemia Vera

Medical Classification Codes: MeSH Code: D011087 | SNOMED ID: 109992005

Diagnosis Threshold: Standard diagnostic criteria for PV (WHO criteria)

Medical Methodology: Managed Access Program / Expanded Access
Optimization Target: Disease control and management of PV symptoms

13. Study Procedures & Methodology

Management Pathway: Patient-specific treatment plan managed by an independent treating physician

Education Protocol (HCP): Novartis-provided suggested treatment guidelines and safety reporting instructions

Education Protocol (Patient): Standard of care counseling by treating physician

Quality Audit Mechanism: Drug supply accountability and pharmacovigilance compliance tracking

14. Observation & Follow-Up Schedule

Total Duration: Variable (until commercial availability, loss of benefit, or discontinuation)

Onsite Visit Cadence: According to the treating physician's standard clinical practice

Remote Monitoring Frequency: N/A (Based on real-world clinical care requirements)

Checklist Review Interval: Periodic safety and drug accountability reporting

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	_____	[DD-MMM-YYYY]				